

The INJECT-RESPECT Tool

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Instructions:

These prompts provide guidance to obtain a history of injection drug use, focusing on infectious and non-infectious harms. The interview should ideally be conversational, with these points acting as a guide, rather than a checklist. For situations where time is limited, focus on the “Quick Interview” column. If time is plentiful, use questions from *both* columns. Focus the history on the past few months or weeks, since that has a greater influence on the current risk of infectious and other complications.

The questions in this tool may elicit feelings of shame, trauma, and stigma among people who use drugs. Many people who use drugs have experienced stigma from healthcare workers and/or in healthcare settings. Additionally, it is important to value people who use drugs as complex, unique individuals. Be curious and treat everyone with respect.

To facilitate an open and respectful conversation, frame the conversation around the health of the patient. If you have information about positive drug toxicology testing, present that first in a matter-of-fact way, rather than leading with questions about drug use that could put patients in the position to feel misled or tricked. To the extent possible, limit the number of clinicians in the room and take steps to ensure privacy. The conversation should directly assess risks and yield opportunities for specific harm reduction counseling and further treatment. See the references below for more information on harm reduction counseling.

INJECT-RESPECT:

Intensity (frequency, amount, quantity)

Network (sharing, lending, buying)

Junk (drug, substance, route, form)

Effect (hit, motivation, feeling)

Cleaning (supplies for reuse, skin)

Tools (syringes, needles, water, filters, cookers)

Reversal (naloxone)

Environment (setting for use, safety)

Site (anatomic location, tissue)

Preparation (cooking technique, loading syringes, tourniquet)

Education

Care (of injection sites)

Technique

	Quick Interview	Detailed Interview
<u>Intensity</u> frequency, amount, quantity	• When was your most recent injection drug use? • How frequently do you usually inject drugs?	• How many times per day (or week) do you inject drugs? • Has the amount used increased, decreased, or stayed the same during the past month? Why has it changed?
<u>Network</u> sharing, lending, buying	• Do you share drugs or equipment with other people? With a sexual partner?	• Do you know if those people have HIV infection? • Do you give used supplies to others?
<u>Junk</u> drug, substance, form	• Which drugs do you inject?	• What formulations of drugs do you inject (<i>e.g.</i>, tablets/pills, powdered vs black tar heroin)?
<u>Effect</u> hit, feeling	• Do you often or have you recently missed a vein or had trouble injecting?	• Do you test your drugs with fentanyl test strips, xylazine test strips, or other tests? • Do you take test-hits of your drugs to determine potency?
<u>Cleaning</u> supplies for reuse, skin	• Do you reuse needles, syringes, filters, or other supplies on yourself? • Do you wash or clean your injection site before injecting? If so, with what (<i>e.g.</i>, soap, alcohol)?	• Do you clean supplies before you reuse, borrow, or lend them? • How do you clean your supplies (<i>i.e.</i>, boiling, bleach)?
<u>Tools</u> syringes, needles, filters	• Where do you obtain syringes, needles, and other equipment?	• What do you use as a filter (<i>e.g.</i>, cigarette filter, pocket lint, sterile cotton)? • How do you dispose of needles and syringes?

	Quick Interview	Detailed Interview
<u>Reversal</u> overdose, naloxone	• Have you ever had an overdose or a near-overdose? • Is naloxone (Narcan) available while you are using? How often?	• Is someone available to administer naloxone (Narcan)? • Do you have naloxone on you currently?
<u>Environment</u> setting for use, safety	• Do you inject alone?	• Where are you when you typically inject drugs (e.g., home, street, supplier's home, shooting gallery, safe injection space)?
<u>Site</u> anatomic location, tissue	• Where on your body do you inject (e.g., arms, legs, groin, neck)? • Do you know how to locate veins and arteries, and how to tell the difference?	• Do you inject in veins (<i>mainline</i>), arteries, wounds, skin (<i>skin pop</i>), muscle (<i>muscle pop</i>, <i>muscle-ing</i>), or other tissues? • What sites have you injected in in the past month? • Where was your last injection?
<u>Preparation</u> cooking technique, loading syringes, tourniquet	• Has your syringe or other equipment typically been used before by someone else to inject? • Do you lick your needle prior to injecting?	• What do you use to dissolve drugs (e.g., water, vinegar, etc.)? • Do you use a syringe to split or mix drugs? • Do you draw blood directly into a syringe filled with powder (instead of dissolving the powder in water first)? • Do you use a tourniquet? • Do you draw blood into the syringe before injecting (<i>booting and jacking</i>)?
<u>Education</u>	• Do you know where to safely obtain clean injection supplies?	• Do you know where to obtain information on safer injection practices? • Have you considered ingesting, smoking, snorting, or booty bumping drugs instead of injecting?
<u>Care</u> of injection sites	• How do you care for your injection sites? • Do you inject into the same site even if it seems to be infected, red, swollen, or painful?	• Do you purposefully preserve any wounds for the purpose of injection into the wound bed or edge? • Do you use antibiotics obtained from somewhere other than a pharmacy? • What do you do if you have an area that might be infected? • Do you use techniques or products to help heal wounds (e.g., lotions, ointments, or soaking)?
<u>Technique</u>	• What injection techniques do you use to prevent injury and infection?	• Are there any broken needles in your body?

Glossary of Terms:

Term	Definition
Backloading	Dividing a drug dose into multiple syringes by loading into the back of an empty syringe after removing the plunger. Also called piggybacking.
Black tar	Form of heroin in a thick, semi-liquid form. Requires acidification for dissolving. Common on West Coast of USA.
Booting	Process of drawing blood into the syringe to dissolve residual drug before reinjecting. Also called “booting and jacking.”
Booty bumping	Rectal administration of drugs, usually in aqueous solution using a needle-less syringe. Also known as <i>boofing</i> .
Coke	Cocaine.
Cook	Process of dissolving powdered drug in water, through heating.
Cooker	Device used to heat drug and water solution, usually small metal basin with a handle.
Crack	Crystalline cocaine.
Dope	Heroin or other opioids.
Get off	Use a drug, especially with desired effect.
Fell out	Variable meanings, including fell down, lost consciousness, or overdosed.
Filter	Material used to remove particulates from a drug and water solution, commonly cotton.
Front-loading	Dividing a drug dose into multiple syringes by loading into the front of an empty syringe after removing the needle.
Harm reduction	Philosophy and practice of minimizing the negative consequences of drug use, even with ongoing use.
Hit	Successful drug administration with resulting effect, or the drug dose.
Hot blood	Arterial blood, which appears bright red (a.k.a. <i>red blood</i>) and may cause a hot sensation if it is injected into.
Honey-hole	A favorite spot on the body to inject, which yields reliable hits.
Injection drug use (IDU)	Preferred term for the practice of consuming drugs through injection, including mainlining or skin or muscle popping. More accurate than “IV drug use.”
Mainline, (-ing)	Intravenous drug injection.
Muscle popping	Intramuscular drug injection. Also known as “muscle-ing”.
Naloxone	Opioid receptor antagonist medication, used to reverse opioid overdoses. Brand name <i>Narcan</i> .
Needle exchange	Program which provides sterile, sealed injection equipment, including syringes, needles, cookers, filters, water, tourniquets, and alcohol swabs. Also called needle and syringe exchange, needle and syringe service program, etc. Does not necessarily require exchange or return of used equipment.
Opioid	Natural, synthetic, and semisynthetic chemicals acting on the opioid receptors, including opium, heroin, morphine, codeine, oxycodone, methadone, and fentanyl.
Person who injects drugs (PWID)	Preferred term for a person who consumes drugs through injections (see “Injection drug use”). A person-first term, which avoids overly labeling a person, in contrast to “IV drug user” or “addict.”
Safe injection site	Facility with medical staff to use drugs under supervision. Also called safer injection site, drug consumption rooms, overdose prevention site, and safe consumption site.
Scramble	Mixture of an opioid (i.e., heroin or fentanyl) and diphenhydramine, lactulose, and/or benzodiazepines.
Shooting up	Injecting drugs.

Shooting gallery	Facility maintained for use of drugs.
Shot	A dose of drug or drug-water solution.
Skin popping	Intra- or subdermal drug injection.
Speed	Amphetamine.
Speedball	Mixture of an opioid and cocaine or amphetamine.
Tester	A drug dose given out by dealers, usually for free, so that users can report back on the potency of the batch. May be unexpectedly potent, causing an overdose.
Test dose	Small dose of drugs taken to test the quality and potency of the larger batch. Can be used to avoid unintentional overdose.
Tourniquet	Rubber or cloth strap to constrict an arm or leg, causing the veins to distend and to facilitate intravenous injection. Also called a tie.

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